Mp10 Web — User Guide

Finding patients, working encounters, and everything the browser can do

Structured Systems · www.structuredsystems.com

03 November 2026

Contents

1 Mp10 Web — User Guide **4**

1.1 Signing in . . . 4
1.2 Getting around . . . 4
1.2.1 What the grids remember . . . 5
1.3 The Dashboard . . . 5
1.4 Finding a patient . . . 6
1.4.1 The Created column . . . 7
1.5 The patient record . . . 7
1.5.1 Demographics . . . 7
1.5.2 The HIPAA signature . . . 7
1.5.3 Insurance . . . 8
1.5.4 Encounters . . . 8
1.5.5 The Flags column . . . 8
1.6 Claims . . . 9
1.6.1 The Status column . . . 9
1.6.2 What you can do with a claim . . . 10
1.6.3 Payments . . . 10
1.7 Remittances . . . 10
1.7.1 The list . . . 10
1.7.2 The check, and its claims . . . 11
1.7.3 Correcting a remittance . . . 11
1.7.4 Posting . . . 11
1.7.5 Printing a remittance . . . 12
1.7.6 Who may do what . . . 12
1.8 Submissions . . . 12
1.8.1 Picking claims on the Claims page . . . 12
1.8.2 Create 837 . . . 13
1.8.3 The Submissions page . . . 13
1.8.4 Sending . . . 14
1.8.5 A claim already submitted . . . 14
1.9 Reports . . . 14
1.10 The grid action bar . . . 15
1.10.1 Working an encounter . . . 16
1.11 Orders — what the patient is in for . . . 16
1.12 Results — what came back . . . 17
1.12.1 Transcribed and scanned results are different, and the list says which . . . 17
1.12.2 If a result will not print . . . 17
1.13 The Modality worklist . . . 17
1.13.1 When the list is empty . . . 18
1.14 Capturing images . . . 18
1.14.1 Scanning a card . . . 19
1.15 Attachments . . . 19
1.15.1 Scanning a document . . . 19
1.15.2 When scanning does not work . . . 20
1.16 Patient signatures from a signature pad . . . 20
1.16.1 If it does not start . . . 20
1.17 Printing encounter forms and labels . . . 21
1.17.1 If it asks you to choose a printer . . . 21
1.17.2 Page numbers, and printing to PDF . . . 22
1.17.3 When printing does not work . . . 22
1.18 Admin → Signature helper . . . 22

1.19 Who made a change 22

1 Mp10 Web — User Guide

This guide is for clinic staff using Mp10 Web in a browser. It does not cover how the system is installed or built — that is the **Mp10 Web — Installation** guide.

Mp10 Web gains screens as they are built, and this guide is revised with them. If a screen here is missing from your menu, either your account has no permission for it or your site has not turned that feature on; if a screen in your menu is missing here, your site is running a newer version than this guide. Last revised **September 2026**, covering the Dashboard, Patients, Encounters, Claims, Remittances, Submissions, Reports, the Modality worklist, attachments, signatures and printing.

Three companion guides cover things this one only points at: **Mp10 Web — Printing** for the print helper, **Mp10 — Signature Helper** for the program that drives both the pad **and the scanner** at your desk, and **Mp10 Web — Signature Pads** for the pad itself.

1.1 Signing in

Go to the Mp10 Web address your office gives you. Enter your username and password and select **Sign in**.

What you can see and do depends on your account. If a screen or button described below is missing, your account doesn't have permission for it — ask your administrator.

Changing your own password. At the bottom of the menu, beside the sign-out button, is a **key** ( ). It asks for your current password and the new one twice. The new password must be at least 8 characters, and your site may require more — the box tells you which. It also has to differ from the one you are replacing, and the two new entries have to match.

Changing it **signs you out of Mp10 Web everywhere**, including the window you are in — that is deliberate, so a password you have just replaced cannot still be in use somewhere you have forgotten about. You are taken back to the sign-in page to sign in with the new one.

Your Mp10 Web password is separate from your Mp10 desktop password. Changing it here does not change the one you use in the desktop applications.

1.2 Getting around

After signing in you'll see a menu down the left. It only lists the areas you have permission to use, so two people may see different menus on the same system. The main areas are **Dashboard**, then **Patients**, **Encounters**, **Claims**, **Remittances**, **Submissions** and **Modality worklist** grouped under a **Records** heading, and **Reports** under its own. There is also an **Admin** heading, and **it starts collapsed** — select it to find **Print helper** and **Signature helper**, which are referred to later in this guide.

Which of those you get depends on the group your administrator put you in — the same groups the Mp10 desktop uses, set in the desktop Admin module. There is no separate web permission to ask for. As a rule of thumb:

If you are	You see
admissions staff	Patients and Encounters, and an encounter's claims read-only — **not** the Claims page, Remittances, Submissions or Reports
billing staff	Claims, Remittances, Submissions and Reports, with Patients and Encounters read-only
transcription	the Modality worklist
an administrator	Patients, Encounters, Claims, Reports, the worklist and the Admin screens — but posting a remittance, sending an 837 and entering an adjustment are billing rights, granted separately

So a colleague who does billing having screens you do not is expected, not a sign that your account is broken. If you need one of them, it is a change to your group in the desktop Admin module, and it takes effect the next time the page loads.

Two entries appear only where the site uses them. Modality worklist is there when the site runs the imaging worklist; where it was never turned on the menu entry is absent, because there is nothing behind it. The **HL7 interface** row on the Dashboard behaves the same way. Neither is a fault or a permission problem — it is the difference between a feature your site does not use and one your account may not see.

Two ways a button can be unavailable, and they mean different things. A button your account may not use is not shown at all. A button you may use, but not on the row you have selected, is shown **greyed** — and hovering it says why, such as *"Select a row first"* or *"This patient already has a HIPAA signature"*. So a greyed button is worth hovering before asking anyone.

Permissions are re-read every time the page loads, so if an administrator grants you something, reloading the page is enough — you do not have to sign out and back in.

The menu can be collapsed to icons with the control at its top, to give the grids more room. Mp10 Web remembers whether you left it collapsed.

1.2.1 What the grids remember

The four big screens — Patients, Encounters, Claims and Modality worklist — all behave the same way, and both kinds of memory below are per person, so two people sharing a computer keep their own.

Column widths and order are remembered for good. Drag a column edge to resize it, or drag a heading to move it, and it stays that way the next time you sign in — on that computer. If you get into a mess, **Reset layout** in the button bar puts the columns back to how they ship. It only affects columns; it does not clear your search or filters.

Filters are remembered until you close your browser. Search a name on Patients, go to Encounters, come back — your search and filters are still there, and the grid is showing what it was showing. Close the browser and they are gone; this is meant to save you retyping during a shift, not to follow you into next week.

The **Modality worklist**'s date range follows a rule of its own, because that box is filled in for you rather than starting empty. **A date range you set yourself is remembered** like any other filter. One you never touched is not — it is reset to today each time, so a browser left open overnight opens on today's work rather than yesterday's. **Clear** puts it back to following the calendar.

1.3 The Dashboard

After signing in, the Dashboard is what you land on first. What you see depends on what you are allowed to see, so a colleague's may have fewer cards than yours:

- **Today** — a quick count of today's encounters, split out by site. If today shows nothing yet — first thing in the morning, or on a weekend or holiday when the office is closed — that's normal, not a fault: the card names the day it's showing and also tells you the last day that actually had encounters, so you can tell "quiet today" from "something's wrong" at a glance.
- **Needs attention** — encounters from the last several days that still need a signature. Use it to catch anything that was missed before it gets old.
- **System health** — a few quick status rows (the Modality worklist row is there only if your account may see the worklist):
  - **Print helper (this workstation)** — whether the print helper program is answering on **the computer you're sitting at right now**, not on the server. It only knows about itself, so if you check this at the front desk and a colleague checks it at their own desk, you can each see a

different answer — that's expected, not a bug. If it shows a job stuck ("stopped responding"), see **When printing does not work**.
  - **Signature helper (this workstation)** — the same thing for the program that drives the signature pad at your desk. Also workstation-local, and also normal to differ from desk to desk: a computer with no pad attached has no reason to be running it.
  - **HL7 interface** — shown only where the site runs an interface. It gives when the last message came in from the lab or imaging interface and how many are still waiting to be processed. It turns red when the interface program itself stops reporting in, and when messages have gone quiet for longer than expected. A large "unprocessed" count on its own is normal for a busy practice and does not turn the row red by itself.
  - **Modality worklist** — how many imaging orders are queued. Seeing "nothing scheduled" here is a normal, everyday answer, not an error — it just means nothing is queued right now.

- **Billing** — four counts over the last 90 days: encounters with no claim ("unbilled"), claims rejected in the window (by the date the rejection was recorded, whatever the encounter's date), and that window's claims that were paid or have no status yet. The first is a count of **encounters**, the other three of **claims**, so an encounter with two claims counts twice in those. Under them the card names the oldest of each, which is the one worth chasing.

  Encounters dated ahead of today are never counted as unbilled. An encounter created for an upcoming appointment carries no charge yet, and the window the card counts ends today, so it falls outside. Where such encounters exist the card says so on its own line — *"16 upcoming encounters pre-created, next 2026-10-09"* — and where there are none the line is absent rather than showing a zero.

- **Print helper** and **Signature helper** — one card each, when you have permission to see them, giving the fuller version of those two health rows: whether the program is answering at this desk, how many jobs it has done and how many failed. Both link through to their own page. Like the rows above, they describe **this computer** — sitting at another desk shows that desk's answer.

1.4 Finding a patient

The patient list shows every patient in a scrollable grid, with search, filtering, sorting, and paging so you can find one quickly among the full list.

- **Search** — type a last name, first name, Social Security number, record number or e-mail address, and press Enter. Partial names work. There are **Search** and **Clear** buttons beside the box if you prefer them.
- **Filter** — narrow the list by sex using the box above the grid, and by city or date of birth using the small funnel in those column headings. The City funnel takes more than one city at once. Every funnel has a **Find...** box and **Apply / Clear** buttons; on a long list like City the Find box is the quick way in.
- **Sort** — select a column heading to sort by it; select again to reverse the order. Not every heading sorts — **Sex**, **Phone**, **Mobile** and **St** and **Signature** have no arrow, because sorting a list this size on those columns would be slow enough to be worse than useless.
- **Paging** — use the controls at the bottom to move through pages of results. The count beside them is the number of patients your search matched, not the number on screen.

Single-click selects a row; double-click opens it. Selecting is what turns on the buttons in the bar at the bottom — most of them act on the row you have chosen. The patient then opens **in a window on top of the list**, so closing it puts you back exactly where you were: same search, same page, same row highlighted.

On patients who share a surname. The list is ordered by last name, and within one surname the order is not alphabetical by first name — with over two thousand patients called RODRIGUEZ, sorting them all every time you opened the screen cost more than it was worth. Use the search box, which looks at first names too, rather than paging through a common surname.

1.4.1 The Created column

The last column, **Created**, is the date the patient record was first made. It sorts, so it is the quickest way to see who was registered recently.

1.5 The patient record

A patient's record has four tabs across the top:

- **Demographics** — name, address, contact details, and the HIPAA signature.
- **Additional** — the further identifying details that do not fit on Demographics.
- **Insurance** — the patient's insurance plans.
- **Encounters** — the patient's visits.

Select a tab to switch to it.

The record normally opens **as a window on top of the patients list**, and **Close** returns you to the list untouched. It can also be opened as a full page of its own — a bookmark or a link someone sent you does that — and in that case there is a **Back to list** button instead, and the tab you are on is part of the address, so refreshing or sharing the link reopens on the same tab.

1.5.1 Demographics

This tab shows the patient's personal information: name, address, phone, date of birth, and similar details.

- The fields are ready to type in as soon as the record opens — there is no Edit button to press first. Select **Save** when you are done.
- **Name and identity**: last name and first name are on one line, with date of birth ("DOB") and Social Security number on the next, and sex and marital status below that. The patient ID card photo sits beside these fields (see "Capturing images" below).
- **Language is no longer editable** on this screen. It is not shown or changed here.
- **Zip code**: if you enter a zip code, the city and state fill in automatically — but only if those boxes were empty. If you already typed a city or state, autofill will not overwrite what you entered.
- **ID card photo**: you can attach a photo of the patient's ID (see "Capturing images" below).
- **Mailing address** sits beside the **HIPAA signature**, with the two address lines stacked and the zip, city and state on the row underneath.

1.5.2 The HIPAA signature

The card to the right of the address shows the signature certifying that this **person** received the HIPAA notices. It is not the same thing as the per-visit signature on an encounter — that one is a receipt for a particular visit, this one is about the patient.

Underneath the signature it tells you **when it was given**. That is the point of the card: a signature on file is not much use if nobody can say when it was signed.

Three things you may see:

- a signature and a date — the normal case;
- **"No HIPAA signature on file for this patient"** — nothing has been captured;
- a signature and **"Signed on an unrecorded date"** — the signature is real, but it predates the system recording signing times. It is not an error and there is nothing to fix.

Capture signature (or **Re-capture signature**) takes a new one from a signature pad. It is pad-only by design — a consent cannot be uploaded from a file, it has to be signed. See **Patient signatures from a signature pad**.

1.5.3 Insurance

This tab shows a list of the patient's insurance plans, plus a form below the list with details for whichever plan you select.

- Select **New** to add a plan, fill in the form, and select **Save**.
- Select an existing plan in the list to view or edit it.
- Select **Delete** to remove a plan.
- Select **Cancel** to discard unsaved changes in the form.
- Each plan can have a photo of the insurance card attached (see "Capturing images" below).

Primary, Secondary, and Historical — every plan is marked as one of these three. Primary and Secondary are the patient's active coverage; Historical means the plan is on file but no longer active. There is no separate "active" checkbox — this Primary/Secondary/Historical setting is what determines whether a plan counts as active.

If you see "This record changed since you loaded it. Reload and try again." — this means someone else (on the web or on a desktop screen) saved a change to this same insurance record while you had it open. This is expected behavior, not an error in the system. **Do not simply try to save again.** Reload the patient's insurance tab first, so you're looking at the current version, then make your change again. Saving blindly on top of someone else's change can create a duplicate record instead of updating the one you meant to.

1.5.4 Encounters

This tab lists the patient's visits (encounters), newest first.

- Select **New** to add an encounter, or select an existing one to edit it. Either one opens in a dialog on top of the patient record, so you never leave the patient's page just to work on an encounter.
- The action bar at the bottom of the grid (see "The grid action bar" below) has an **Active on/off** button. Inactivating an encounter does not delete or hide it — it stays in the list, shown in grey, and you can select it again later and reactivate it.

Why "newest" means highest encounter number. Encounter lists are ordered by encounter number rather than by admission date. A small number of encounters brought in from older systems years ago carry admission dates that are plainly wrong — a year like 9201 or 4208 — and ordering by date put those at the very top, so the first thing you saw was a screen of bad data. Encounter numbers are issued in order, so they give the same "newest first" without that. The odd dates are still in those old records; they are simply no longer what greets you.

You can still sort by **Admitted** by selecting that heading, which is occasionally what you want — just expect the strange ones at the end.

1.5.5 The Flags column

Both encounters grids carry a narrow **Flags** column. It shows small marks telling you, at a glance, what an encounter already has:

Mark	Means
a pen nib	a **signature** is on file for this encounter
a lined document	this encounter has **results**

An encounter with neither shows nothing at all. That is deliberate: with no placeholders to read past, the rows that do carry something are the ones your eye lands on when you run down the column. An encounter can show both marks, one, or none.

Hover over a mark and it will tell you what it is, so you do not have to remember which shape is which.

The column does not sort. Both marks are worked out after the page of encounters has already been fetched, so a sort arrow there could not do anything, and one that silently did nothing would be worse than none.

1.6 Claims

Claims, under Records between Encounters and the Modality worklist, lists every claim raised on any patient. It opens showing claims with a **service date in the last 30 days**; change the From and To dates to look further back, or press **Today**. **Clear** puts the 30-day window back.

Finding one claim. The search box takes a claim number (`CL26-…`), an encounter number (`EN26-…`) or a patient's record number, and finds it regardless of the dates. It does not search by name — for that, find the patient and open their record. If you type something it does not recognise it says so above the grid rather than showing an empty list.

Filtering by payer. The Insurance heading has a small funnel; select it to tick one or more payers. The heading is highlighted while a filter is on; Clear removes it.

1.6.1 The Status column

The first column shows a coloured flag:

Flag	Means
green	paid in full — a payment is on file and the insurance balance is zero
yellow	partially paid — a payment is on file and a balance is still owed
black	overpaid — the insurance balance is below zero
red	rejected — no payment, and the payer's latest answer is a rejection
white	no payment yet, but the payer has answered something else
blue, orange, pink, purple	no payment, and the payer's latest answer is one of the four statuses your site defines for itself
none	no payment and no status yet

Money outranks status. The first three colours are worked out from the claim's CPT lines and its payments, the same figures the balance columns show, so a flag and its row always agree. A claim that was paid in part and later rejected stays yellow, because the balance beside it is real. Only where nothing has been paid does the payer's latest answer set the colour.

The four site-defined colours are the statuses your practice names itself — they read with your own wording, which the desktop Admin module sets, so "Blue" may be "Appealed" at one site and something else at another.

Hover over a flag to read its meaning. **Click a flag** to see the claim's status history — every answer the payer has sent, newest first. The first column is **Recorded**: the date *and time* the answer was written down, since several answers often arrive on one day and the order matters; hover it to see the payer's own date, which carries no time. Where the payer's file named no insurance, the claim's own is shown rather than a dash.

The Status heading has a funnel like Insurance's: tick one or more colours ("No flag" is a choice too) and the list narrows to claims in that state across the whole window, not only the page on screen. Filtering by colour takes a moment longer than the other filters — about a second for a month of claims — because each claim's payments are added up to answer it.

Payer status shows the newest entry in the claim's status history and, on hover, the date it was received.

1.6.2 What you can do with a claim

Select a row and use the action bar: **Open** shows the claim (and lets you edit it if you may edit encounters), **Preview** shows the CMS-1500 as a PDF, **Print** sends it to the FORMS printer, and **837** shows the electronic claim exactly as it was transmitted — one entry per send, newest first, with the segments the payer received. A claim that has never been transmitted says so.

Two boxes fill themselves on a new claim. Contract starts as the member number on the patient's insurance card for the payer being billed, and follows the payer if you change it before saving; on a saved claim it is left alone. Performing physician is picked from the staff list rather than typed: the staff set up for the claim's payer come first, then everyone else, each shown as number, name, NPI and payer.

Opening, printing and raising a claim need the billing permission, which the desktop Admin module grants to the **BillingUsers** group; a button you lack the permission for explains that on hover. The Claims page itself appears only for that group. Admissions staff still see an encounter's claims list from Encounters, read-only. Raising a new claim is done from the encounter, not from here — see "Working an encounter". Adjustments (a non-zero Ins adjust or Pat adjust on a payment line, or a negative amount in any of the four money columns) additionally need the AdjustmentAuthUsers group. That group grants only the adjustment permission, so an operator who posts adjustments needs BillingUsers as well — AdjustmentAuthUsers on its own does not open the Claims page.

1.6.3 Payments

Select a claim and press **Payments**. The top grid is every payment posted to the claim, newest first, with a total under each money column. Below it, if you are in the BillingUsers group, is a grid to post new payments:

- **One line per CPT.** Pick a CPT from the claim's own lines or type one (a payer sometimes pays under a different code; the page warns, it does not refuse).
- **Insurance** may be blank — that is a patient payment.
- **Date paid** starts as today and can be changed per line.
- A line with every amount zero must carry a **check #** — it records a rejection. A line with no amounts and no check is not saved.
- **Ins adjust / Pat adjust** need a **Reason**, and posting them needs the AdjustmentAuthUsers group. Without it the cell is marked and Save stays off.
- A **negative amount in any column** — a reversal of a payment posted in error, a refund — is an adjustment too: it needs the same Reason and the same AdjustmentAuthUsers group.

The two balance panels show the insurance and patient balances now and what they would be after Save. Save posts the lines, brings the claim's stored totals and status in line, and the Claims page behind reloads — the Status flag may change.

Co-insurance assignment and the complementary claim are still posted on the desktop.

1.7 Remittances

A remittance is one payer's check and everything it pays: several claims, each with its own CPT lines. Most arrive as an electronic 835 file that AutoTasks loads overnight; some are read out of a PDF the payer sent; and one can be keyed here by hand. This page is where you look at them, correct them, and post them to the claims.

It needs the billing permission to open, and posting needs one more — see **Who may do what**, below.

1.7.1 The list

The top half is one row per remittance, over a date range that works like the other screens: the last 30 days to begin with, changeable, with **Today** and **Clear**. The search box finds a **check number**; unlike the Claims search, it keeps the date range, because check numbers repeat between payers and years.

Two funnels sit in the headings: **Payer** and **Posted**.

The **Posted** column is a dot, so the state is readable at a glance:

Dot	Means
green	every line of the check has been posted
red, "Unposted"	none of it has been posted
red, "3 unposted"	some of it posted and some did not — the count is how many lines are still out

The line under the grid gives the window's totals: how many remittances, and what they paid, adjusted, took as deductible and assigned.

1.7.2 The check, and its claims

Select a remittance and the strip below fills in with the check's facts — payer, check number, check date, received, deposit, check total — then **Lines paid** and **Difference**. The difference is the check total less what the lines add up to; it is red when it is not zero, which is the number to settle before posting.

While a remittance is unposted, the received date, deposit date and check total can be edited in the strip. Once any line is posted the whole check becomes read-only, and the strip says who posted it.

Under the strip is the check's claims. Each row is one claim: patient, ICN, the money it was paid, and its flag. Expand a claim to see its CPT lines. The **Find claim or patient** box filters this tree as you type, by claim number or patient name, which is how you find one claim on a check that pays four hundred.

The flags here are the payer's own answer for that claim, and the legend above the tree names them: balanced, rejected, pending, duplicate, claim not found, reversal, zero paid. **Claim not found** matters most: it means the claim the payer names is not in this dictionary, so posting will skip it — fix the claim number first or remove the line.

Click a claim number to open that claim's own record, read-only, without leaving the remittance.

1.7.3 Correcting a remittance

Everything here is possible only while the remittance is unposted, and only with the billing permission.

- **Add claim** finds a claim by number, encounter or record number, shows what it was billed, and gives you a line per CPT to fill in.
- **A line can be edited where it sits**: click a Paid, Adjusted, Deductible, Assigned or Reason cell in the tree and type. The claim's totals and the strip's difference follow as you type. **Save** writes them.
- **Delete claim / Delete line** removes what is highlighted, after asking. A claim takes its lines with it; the last line of a claim takes the claim; the last claim takes the remittance. Only this remittance's rows are removed — the claim record itself is never touched.
- **Delete remittance** removes the whole check, and only while nothing on it is posted.

New remittance keys one from scratch: fill in the payer, check number and dates, add the claims, and Save.

1.7.4 Posting

Post writes the remittance to the claims: each line becomes a payment, the claims' totals and status are brought in line, and the lines are marked posted. It asks first, and the confirmation is worth reading:

- It shows the check total against the lines paid. **If they differ, posting needs you to tick "post anyway"** — that is the safety catch for a check that was keyed short.
- **Post deductibles** is on by default and matches the desktop.

- It names how many claims will post and how many are **not found**. Those stay unposted, which is why the check then shows a red dot with a count rather than a green one. Fix them and post again — the second pass only touches what is still out.

Posting cannot be undone from the browser, and a posted remittance cannot be edited or deleted here. A remittance whose claims carry **assigned** amounts prints a note saying the complementary claims have not been created — that step is still the desktop's.

1.7.5 Printing a remittance

Print opens the remittance in a new browser tab, laid out for paper: the check's facts, then each claim with its CPT lines, and **under each line the payer's own explanation of what it adjusted** — the code and the sentence that goes with it, which the grid has no room for.

Highlight a claim first and the button reads **Print claim**: the tab then holds that one claim, and its total line says how many claims are on the whole check.

The tab has its own **Print** button, which opens the browser's print dialog. That dialog is also where you save it as a PDF — choose **Save as PDF** as the destination instead of a printer.

Page numbers come from the print dialog, not from the page. Browsers do not let a page number its own sheets, so "Page 2 of 9" is printed by the browser itself, along with the date and the document name, when **Headers and footers** is ticked. It is ticked by default, under **More settings** in Chrome and Edge. If you turn it off the numbers go with it; every page still carries the payer and check number across the top, so a loose sheet can be placed.

A long check paginates properly: the column headings repeat at the top of each page, a claim's heading never sits alone at the foot of one, and a line is never separated from the explanation beneath it.

1.7.6 Who may do what

To	You need
open Remittances and look	the billing permission (**BillingUsers**)
key, edit or delete an unposted remittance	the same
post a remittance	**remits.post** as well — your administrator grants it in the desktop Admin module
print	the billing permission

A button you may not use is not shown at all, so a Remittances page with no **Post** button means your account does not have that one.

1.8 Submissions

Submitting claims electronically is a hand-off in stages: **Claims** is where you pick which claims go out, **Create 837** is where you agree to what will be built, the file itself is built in the background by the AutoTasks service, and **Submissions**, next to Remittances under Records, is where you watch all of that happen and, when a file is ready, send it. Every action on this page and the Create 837 button on Claims need **claims.submit**, which your administrator grants in the desktop Admin module — the billing permission alone is not enough.

1.8.1 Picking claims on the Claims page

A tick-box column appears on the left of the Claims grid. Tick a row, or:

- **Shift-click** a second row to tick every row between the two.
- **Ctrl-click** a row to toggle just that one, without disturbing the rest.

- The box in the column heading ticks or clears **the page** — it can only reach the rows on screen.

The selection survives paging. That is the point of it: a batch can span many pages of claims, and moving to the next page does not lose what you already ticked on this one. A bar appears above the grid as soon as anything is picked, showing how many claims that is.

If the filter you have set (dates, payer, status) matches more claims than are ticked, the bar offers **Select all *N* matching** — meaning every claim the current filter finds, not only the page you are looking at. Use it to submit an entire batch (for example, everything unbilled for one payer) in one motion instead of paging through and ticking each row.

Clear selection empties the bar. **Create 837...** opens the dialog that actually queues the work, and is greyed out for anyone without **claims.submit**.

1.8.2 Create 837

The dialog shows what would be built, before anything is written: **one file per payer** in your selection — a mixed batch is split automatically rather than refused. The table lists each payer with how many claims and how much is billed, and **every number on this screen comes from the server**, not from counting the ticks in the browser, so what you agree to is exactly what gets queued a moment later.

Before that table is filled in, each claim is checked. Some findings **block** a claim — it is left out of the count and the file entirely:

Blocked because…
the claim has no CPT lines — nothing would be billed
the payer has no provider NPI on file — check the insurance definition
the claim has no insurance member number
the patient's sex is not recorded as M or F
the patient's date of birth is missing, or later than the service date

Other findings only **warn** — the claim still goes in the file, but you are told:

Warned because…
the patient-to-insured relationship on the claim is not one the 837 accepts
the claim's remarks are longer than 80 characters and will be truncated
the claim was already put in an earlier 837 file

Blocking findings are listed first and are why the count above them may be smaller than what you ticked; the dialog says how many of the selection could not be sent. Warnings are listed too, so a claim that goes in flagged is not a surprise later.

Press **Create the file** (or **Create *N* files**, one per payer) to queue the work. Nothing is sent at this point either — the files are only queued for the AutoTasks service to build — and the dialog then takes you to the Submissions page to watch it.

1.8.3 The Submissions page

Every job you and your colleagues have queued is listed here, newest first, and refreshes itself on its own while anything is still moving — you do not need to press anything to see a job's status change.

Status	Means
Queued	waiting for AutoTasks to pick it up

Status	Means
Building	AutoTasks is building the file now
Built	the file exists; the Result column names it
Failed	AutoTasks could not build it; the Result column says why
Cancelled	you (or a colleague) cancelled it before it was built

A job sitting at Queued for a minute or two is normal, not stuck. AutoTasks looks for new submissions every couple of minutes, not continuously, so a freshly-created job waits for that next pass before it starts building. A banner above the grid says as much while anything is still waiting.

The **Waiting** column says how long each job has been in the state it is in, and stays blank once a job has finished. **If anything has been queued or building for longer than about ten minutes, an amber warning appears above the grid** and names what to check — usually that the AutoTasks service is not running on the server, that 837 processing has been switched off for the site, or that the dictionary was updated without running the admin module's structure update. That warning is the difference between "waiting is normal" and "nothing is coming": a job that has stopped part-way through building or sending is not retried on its own, and your administrator has to clear it (the AutoTasks install guide says how). Tell them rather than queueing the same claims again — a submission that really did go out and is then sent a second time bills every claim in it twice.

Cancel is offered only on a job that is still Queued — once AutoTasks has started building it, it can no longer be pulled back, and the button is greyed out with that explanation. If you press Cancel just as AutoTasks happens to pick the job up, the page tells you plainly that you were too late rather than treating it as an error.

1.8.4 Sending

Building a file and sending it to the clearinghouse are two separate, deliberate actions — nothing here is transmitted automatically. Once a job reads **Built**, select it and press **Send to clearinghouse**. The confirmation names exactly what you are about to send — the payer, how many claims, and the file — because this cannot be undone from here once it has gone. Pressing it does not send the file itself; it asks AutoTasks to send it on its next pass, the same way a build is queued, so a row reading "Queued to send" for a short while is normal too.

The **Sent** column tracks the send the same way Status tracks the build; the **Result** column shows the outcome, and where the clearinghouse itself answered, hovering over the Sent column shows its reply.

1.8.5 A claim already submitted

Ticking a claim that was already sent in an earlier 837 does not stop you — it is one of the pre-flight's warnings, not a block — because resubmitting a corrected claim is a normal thing to do. Every send is recorded on the claim itself: open the claim on the Claims page and press **837** to see every transmission, newest first, with the segments exactly as they were sent. A claim that has never been transmitted says so there instead.

1.9 Reports

Reports in the menu opens a page of report cards, grouped Financial, Productivity and Coding. It needs the reports permission, which the desktop Admin module grants with the billing group.

Five reports run today:

- **Ageing of Accounts** — what is owed, by how long it has been owed.
- **Payments + Adjustments** — what came in over a period, and what was written off.
- **Billing Report** — what was billed over a period.
- **Periodical Summary** — the period's totals in one sheet.

- **Claims Status** — claims by the payer's latest answer.

The rest of the cards are greyed and marked as not built yet. They are named so you can see what is coming rather than wonder whether you have lost a permission.

Each report works the same way: choose the parameters on the left, **Run**, and the result appears with a chart where one helps and a grid under it. **Export** saves the grid as a spreadsheet; **Print** opens the browser's print dialog, where the destination list also offers **Save as PDF**. As with the remittance print above, page numbers come from the dialog's **Headers and footers**, not from the report.

1.10 The grid action bar

The patients list and both encounters grids (the standalone Encounters list and the patient's own Encounters tab) each have a row of buttons in the bottom-left of the grid, next to the paging controls. Most buttons only work once you've selected a row — select a row first, then choose the action.

On the Patients list, the buttons are:

- **New** — start a new patient record. Works without selecting a row.
- **Edit** — open the selected patient's record.
- **Encounters** — open the selected patient's encounters in a window over the list, without leaving it.
- **Claims** — open a read-only list of the patient's claims.
- **Notes** — open the patient's notes. Unlike Claims, Audit log, and Appointments, you can add a new note here (type it in the box and select **Add note**) — you cannot edit or delete an existing note.
- **Audit log** — a read-only history of changes made to this patient's record, on the web or on the desktop.
- **Appointments** — a read-only list of the patient's appointments.
- **Attachments** — documents held against the patient: consents, referrals, results. Upload one, or scan one on the spot. See **Attachments**.

Each of these opens in its own dialog on top of the patients list, so searching or paging the list underneath is undisturbed.

On an Encounters grid (the standalone list and the patient's Encounters tab), the buttons are:

- **New** — start a new encounter. Works without selecting a row. **Only on the patient's Encounters tab**, not on the standalone Encounters list. An encounter belongs to a patient, and the standalone list has no patient in hand — so it is created from the patient's own record, where you can see whose visit you are creating. The desktop works the same way.
- **Edit** — open the selected encounter.
- **Orders** — record what the patient is in for. See **Orders**.
- **Results** — the results that have come back for this encounter, and printing them. See **Results**.
- **Active on/off** — **on the patient's Encounters tab only**, not on the standalone encounters list. It flips the selected encounter's status, and asks you to confirm. An inactivated encounter is greyed out in the list, not removed, and can be reactivated the same way. (The encounter form also has **Closed** and **Inactive** checkboxes, which change the same thing without asking.)
- **Sign** — see **Patient signatures from a signature pad**.
- **Print Enc**, **Label**, **Preview Enc** and **Preview Lbl** — see **Printing encounter forms and labels**.

The button names are short because the bar is narrow: **Enc** is the encounter form, **Lbl** the label. **Print** sends it to the printer; **Preview** shows it on screen first.

Every one of these opens **in a window on top of the grid**, and the grid is untouched behind it — same filters, same page, same selected row when you close. That applies to patients and encounters alike, from either grid.

If you reach an encounter as a full page instead — a bookmark, or a link somebody sent you — the **Back to list** button takes you to the encounters list.

Reset layout, on the patients list, the standalone encounters list and the worklist, restores the grid's columns to their original widths and order. It leaves your search and filters alone.

1.10.1 Working an encounter

Two fields on the encounter form check what you type against the practice's own lists rather than accepting anything.

Doctor. Type an id, or a surname, or a first name, and a short list of matches drops down — choose one with the mouse, or the arrow keys and Enter. Click the small arrow at the right of the box to browse the whole list instead of typing. The field beside it then shows **who that doctor is**, read-only. With nearly ten thousand doctors on file, an id on its own tells you very little, and this is how you confirm you picked the right one. Upper or lower case makes no difference to the search.

If you type an id that does not exist, the box turns red. Nothing stops you saving — it is a warning, not a barrier.

Admitting code. The same control, against the ICD-10-CM list. Type a code like `E11` to see the codes under it, or type words like `diabetes` to search the descriptions. The **Admitting description** beside it is filled in from whichever code you choose and cannot be typed into — a description that disagreed with its own code would be worse than none.

Leaving it blank is perfectly valid, and normal: this field has not been used before, so almost every existing encounter has it empty. It is checked only if you put something in it.

Only codes that can actually be billed are offered. The ICD-10 list contains group headings and parent codes as well — `E11` is a heading, `E11.00` is a real code — and offering the headings would invite choosing something that cannot be billed.

1.11 Orders — what the patient is in for

An encounter can carry one or more **orders**: the items from your practice's catalogue that the patient is being seen for. Why they are worth entering, how they pre-fill the claim, print as a label and travel to other systems by HL7 is in the **Mp10 Orders** guide. You can open Orders from three places, and all three lead to the same screen:

- the **Orders** button on the patient's Encounters tab,
- the **Orders** button on the standalone Encounters list,
- the **Manage orders** button on the encounter itself.

Orders are only available once an encounter has been saved. On a brand-new encounter, save it first and the Orders card appears.

The screen has two lists side by side. On the **left** is your catalogue — every item you can order. On the **right** is what this encounter has already been ordered. The arrows between them move items across:

- select an item on the left and choose → to add it,
- select an order on the right and choose ← to take it off again.

Nothing is saved until you choose **Save orders**. Until then you are only building up the list, and **Close** discards the lot. After saving you'll see a short summary of what happened — for example "Saved: 2 added, 1 removed".

To find something in a long catalogue, type part of the description or the CPT code in the search box above the left-hand list and select **Search**. Clearing the box and searching again brings the whole catalogue back.

An order that has been transcribed cannot be removed. Once a transcription has been written against an order, that order is part of the patient's clinical record, and the system will not let you take it off the encounter. These orders are shown greyed out and in italics in the right-hand list, and choosing ← on one tells you why instead of removing it. If such an order really is wrong, the transcription has to be

dealt with first — the order can then be removed normally. You can still add and remove other orders on the same encounter as usual.

If you ever see an order described as **"(catalogue entry missing)"**, the item it was ordered from has since been deleted from the catalogue. The order is still real and still on the encounter — it is shown this way deliberately, rather than hidden, so nothing disappears from the record without you knowing.

1.12 Results — what came back

Orders are what the patient came in for. **Results** are what came back: the reports, readings and studies recorded against that encounter.

Select an encounter on either Encounters grid and choose **Results**. A window opens listing every result on that encounter, newest first, showing the case type, the version, the ordering and performing doctors, and when it was created.

Select a result, then:

- **View** — opens it on screen, in a new browser tab.
- **Print** — sends it to the forms printer at the computer you are sitting at.

Nothing here changes a result. This window shows and prints them; results are still produced and edited in the desktop application.

There is also a **Signed** column. On this practice's data it is blank on every result, because the signing step it reports has never been used here — it is shown rather than hidden so that it starts telling you something the day that changes. An empty Signed column does not mean a result is unfinished.

1.12.1 Transcribed and scanned results are different, and the list says which

The **Kind** column tells you how a result was produced, and it changes what you get:

- **Transcribed** — the report was typed. **View** and **Print** lay it out on the practice's own report form, with the patient, the encounter and the doctors filled in.
- **Scanned** — the report arrived as a scan or an imported file, so the result *is* that document. **View** shows you the file itself.

Most scanned results are TIFF files, which browsers cannot display in a tab. When that happens the tab tells you so and offers the file to download instead, named properly so it opens in your usual image viewer. That is not a fault — it is the format the document was filed in.

1.12.2 If a result will not print

- **"No printer whose name contains FORMS..."** — this computer has no printer set up for forms. You'll be offered the list of printers to choose from; pick the one you want, and you can ask to be remembered so you are not asked again.
- **Nothing happens, or the helper is not answering** — the small Mp10 helper program that does the printing is not running at your desk. Same program, and same fix, as for encounter forms and the signature pad. See **When printing does not work**.
- **A message naming your printer** — that text came from the printer's own driver. Fix what it names and try again.

Whether an encounter has any results at all is visible without opening this window: see **The Flags column**.

1.13 The Modality worklist

Modality worklist in the menu shows the imaging orders queued for the scanners — the same queue the imaging equipment itself reads when it asks "what am I scanning today". It is a read-only view: nothing here is edited from the browser.

Each row is one scheduled procedure, with the patient's name, the accession number, the modality (CR, CT, US and so on), the procedure, and when it is scheduled for.

> **Modality is blank on every row today.** The field it comes from is empty in every dictionary so far, so the column shows - and its funnel opens on *"Nothing to choose from yet."* That is a data question for your administrator, not a fault in this screen.

Source and Station. Source is the site the encounter belongs to. Station is the scanner the entry is routed to — the AE title your administrator mapped for that site and modality under Imaging Stations in the desktop Admin module. A dash in Station means no mapping exists for that pairing; the entry is still in the worklist, but a scanner that filters by its own AE title will not see it. That is the first thing to check when a machine says its list is empty.

- **Search** by patient name or accession number. Case does not matter — type `rivera` or `RIVERA`, whichever is quicker.
- **Status** is **Scheduled** by default. **Cancelled** shows what has been called off, and **All** shows both.
- **Modality** and **Source** are funnels in their column headings, and each takes more than one at once — CT and MR together, say, or two sites.
- **Dates** default to today, plus and minus a day, so a procedure scheduled late last night or early tomorrow is still in view. **Change them and your window is remembered**, like every other filter, until you close the browser. Leave them alone and they follow the calendar instead of freezing — a browser left open overnight opens on today, not on yesterday. **Clear** returns them to following the calendar.
- **Auto-refresh** keeps the list current without you doing anything. It is on by default; the tick box turns it off.

Auto-refresh deliberately holds back in two situations, so it cannot pull the screen out from under you: while you have a row selected, and while the browser tab is in the background. Select nothing and it resumes.

1.13.1 When the list is empty

An empty worklist is usually an ordinary answer, and the message says which kind it is rather than making you guess:

- **nothing scheduled in this window** — the normal quiet case;
- **nothing matches your filters** — you have narrowed it to nothing, so widen the dates or clear the search;
- **nothing in the table at all** — this one is worth reporting, because it can mean the process that fills the worklist has stopped rather than that there is genuinely no work;
- **the worklist is not enabled in this dictionary** — different from the one above, and not something waiting will fix. It needs an administrator.

The **Mp10 AutoTasks** guide covers what fills this table and how it is turned on, which is an administrator's job rather than a desk one.

1.14 Capturing images

Both the patient ID card (Demographics tab) and insurance cards (Insurance tab) let you attach a photo. There are three ways to get one in.

- **Scan** — if a scanner is attached to the computer you are sitting at, put the card on the glass and select **Scan**. See below; this is the quickest of the three and the one to prefer for a card.
- **Capture / Upload**, on a desktop computer, opens a file picker. Choose an image you've already scanned into a folder, the same scan-to-folder process your office already uses.
- **Capture / Upload**, on a phone or tablet, can open your camera directly, so you can photograph the card on the spot.

Accepted formats are JPEG and PNG, up to 4 MB. Large photos are automatically shrunk before they're uploaded, so a full-size phone photo is fine — you don't need to resize it yourself. Select **Replace** to swap

out an existing image, or **Delete** to remove it.

1.14.1 Scanning a card

Select **Scan** and a *Scan a card* window opens. Put the card on the glass, press **Scan**, and it appears in the window. Press **Attach** to keep it or **Cancel** to throw it away.

Two things happen to a scanned card that are worth knowing:

- **It is cropped to the card.** You do not need to line it up in a corner or trim it afterwards — the whole page is scanned and the card is cut out of it, the same as the Mp10 desktop does.
- **It is one page, and it replaces what is there.** These fields hold a single image, so scanning a card does the same thing as *Replace*. There is no second side.

If the card comes back **uncropped** — the whole sheet of glass, card somewhere in the middle — the usual reason is the **scanner lid**, left open or too dark to tell apart from the card. Close it and scan again. It is still a usable image either way, which is why you get it rather than an error.

No Scan button, or a greyed-out one, means the small Mp10 helper program is not running on your computer, or there is no scanner attached to it. It is the same program the signature pad uses — see *If it does not start*, below.

1.15 Attachments

Beyond the ID and insurance cards, a patient can hold **attachments** — consent forms, referrals, results, anything scanned or received as a file. Select the patient on the Patients list and choose **Attachments** from the action bar.

From there you can **Import file...** for a file you already have, or **Scan...** one on the spot from a scanner attached to your computer.

1.15.1 Scanning a document

Select **Scan...** and a window opens listing what your computer can scan with. Where the scanner offers a choice, you can set:

- **Scanner**, if more than one is attached.
- **Source** — the glass, or the feeder, on a device that has one.
- **Resolution** and **Colour**.

Some scanners insist on asking for those settings themselves. When yours does, those choices are missing here and the window says so — the scanner's own window will appear when the scan starts, on your screen.

Then:

1. Put the first page on the scanner and press **Scan**. It appears as a thumbnail after a few seconds.
2. To add more pages, put the next one on and press **Scan another page**. Repeat for as many as you need.
3. Press **Attach** when you have them all. With more than one page the button says so — *Attach 3 pages as a PDF*.

One page is attached as an image. Several pages become one PDF. You do not choose; it follows from how many you scanned.

Underneath the last page is a line reporting what came off the scanner and what was stored — *"2550×3507 from the scanner (25.6 MB) stored as 1163×1600 JPEG quality 75 (351 KB)"*. That is not an error. Every scan is shrunk to a sensible size before it is stored, which is why a 300 DPI colour page does not produce a 25 MB attachment, and it is why choosing a very high resolution mostly costs you scanning time rather than buying you a bigger file.

1.15.2 When scanning does not work

- **The Scan... button is greyed out** — the helper program is not running on your computer. Same fix as the signature pad; see *If it does not start*.
- **"...can see no scanner"** — the helper is running but nothing is attached, or the scanner is off or asleep. Switch it on, check the cable, and press **look again**.
- **A message naming your scanner**, after a pause — that text came from the scanner's own driver. Paper jam, lid, cable. Fix what it names and scan again.
- **"That scan is N MB, over the 4 MB limit"** — too many pages in one document. Attach the ones you have and scan the rest as a second attachment, or choose a lower resolution and scan again.
- **It is slow** — a page takes several seconds, and your computer's helper does one thing at a time. During a scan the signature pad at that same desk will wait its turn.

Anything that survives all that is a job for whoever looks after the computers, and it is covered in the **Mp10 Signature Helper** guide — though **Admin → Signature helper** in this application answers most of it without leaving the browser, including which scanners this desk can see — which, despite the name, is the guide for scanning too. It is the same program.

1.16 Patient signatures from a signature pad

If a **Topaz signature pad** is attached to your computer, you can take the patient's signature straight from it, for an encounter or for the patient's record.

The patient does everything on the pad. They never look at the browser. The consent text, the options and the buttons all appear on the pad's own screen, exactly as they do in the desktop application — so a signature taken here and one taken from Mp10 are the same thing, recorded the same way.

Where to start it:

- **An encounter** — select it on either Encounters grid and choose **Sign**; or open the encounter and use **Capture on pad** in the **Signature** panel.
- **The patient's record** — open the patient, and on **Demographics** use **Capture signature** (or **Re-capture signature** where one is already on file).

What happens then is the same either way. A small window belonging to the signature helper comes to the front on your computer, and the pad wakes up. The patient reads the notice on the pad, pages through it if it is long, taps the option they agree to if options were set up, signs, and presses **OK** on the pad. **You then confirm it on the signature helper's own window at your desk** — that small window is where the signature is accepted, and the button in the browser says so while it waits: *"Signing on the pad — then confirm on this workstation"*. It is stored at that point. **There is nothing to press in the browser while this is going on** — it waits, and tells you when it is done.

If the patient presses **Cancel** on the pad instead, nothing is stored and the encounter or record simply stays unsigned.

Re-capturing replaces what was there. On a patient record that already has a signature, the button says *Re-capture signature*, and the new one takes the place of the old, along with the notice and the option that went with it.

1.16.1 If it does not start

The signature helper is a small program that runs on **your own computer** — it has to, because the pad is plugged into your desk, not into the server. It is **not** a Windows service and does not start with Windows; it starts when you sign in. If it is not running, or the pad is not attached, Mp10 Web says so and nothing is stored.

It is the same program that does the scanning, despite the name. So a desk where the pad will not wake up is usually a desk where the *Scan* buttons are greyed out too, and one fix covers both.

Two things worth knowing:

- **It is Windows-only.** From a Mac, tablet or phone there is no pad and no helper, and pad capture is not available. The encounter's **Signature** panel still lets you attach a signature that arrived some other way, by upload or from the camera, and that works everywhere.
- **Check the Dashboard.** The **Signature helper** card says whether the program is answering at the desk you are sitting at. If it is not, that is the thing to fix first, and it is usually a matter of starting it.

Anything beyond that — installing it, the pad drivers, or a capture that stopped responding — is in the **Mp10 Signature Helper** guide, and is a job for whoever looks after the computers rather than something to sort out at the desk.

1.17 Printing encounter forms and labels

Printing happens on **the printer attached to the computer you are sitting at**, using the same templates as the desktop application — so a label printed here and a label printed from Mp10 come out identical.

Select the encounter on either Encounters grid, then:

- **Print Enc** — the encounter form, to this computer's `FORMS` printer.
- **Label** — the encounter label, to this computer's `LABELS` printer.
- **Preview** — exactly what would have been printed, opened as a PDF in a new browser tab, with nothing sent to a printer.

The printer is normally chosen automatically and nothing asks you which one: the form printer is the one whose Windows name contains `FORMS`, and the label printer the one whose name contains `LABELS`.

1.17.1 If it asks you to choose a printer

Where the printers at a desk are not named that way, printing cannot guess, so it shows you the printers this computer has and asks which to use. Pick one and it prints. This is the normal way to work at such a desk — it is not an error, and nothing has to be set up first.

Two things worth knowing when that window appears:

- **"Always use this printer for forms/labels on this workstation"** stops the question coming back. It is a setting for **the computer**, not for you: the next person who sits here gets the same printer. Leave it unticked for a one-off print at somebody else's desk.
- **If the printer you want is not in the list**, choosing another one will not help — the list is every printer the **print helper's own account** can reach, which is not the same as the printers you see when you are signed in — a printer added for one user does not appear there. That one needs installing, which is a job for whoever looks after the computers. Ticking the box is also not how you undo a wrong choice later; ask them to clear it.

Once a printer has been remembered, **Admin → Print helper** shows which one is being used and says that it was chosen here rather than found by name.

Preview is worth knowing about. Use it to check a form looks right before committing a sheet, or when a label came out wrong and you want to see whether the problem is the data or the printer. It is the same template and the same data as printing, not an approximation. If nothing opens, your browser has blocked the new tab — allow pop-ups for this site; the app tells you when that has happened rather than appearing to do nothing.

The Print button on the Claims dialog is not finished. It reports that claim printing is still to be built. Print claims from the desktop application in the meantime.

Designing and editing the report templates stays in the desktop application, deliberately: a template is shared by everyone in the practice, and editing one is a deskbound job.

1.17.2 Page numbers, and printing to PDF

Two different printing paths exist in Mp10 Web, and they behave differently.

Encounter forms, labels, claims and results are printed by the print helper at your desk, from the same templates the desktop uses. They are laid out by Mp10, and anything printed on them — page numbers included — is part of the template.

Remittances and reports are printed by the browser itself, from a tab. There the **page numbers come from the browser's print dialog**: browsers do not allow a page to number its own sheets, so "Page 2 of 9" is added by the browser, along with the date and the document name, when **Headers and footers** is ticked under **More settings**. It is ticked by default in Chrome and Edge. The same dialog is how you make a PDF of either: choose **Save as PDF** as the destination.

1.17.3 When printing does not work

Nothing is ever half-printed: when it fails, nothing was sent, and the message on screen names the cause. The commonest one — no printer named for `FORMS` or `LABELS` — is no longer a dead end: you are offered the list instead, as above. What is left is usually the print helper not running on that computer, which is a setup task rather than something to fix at the desk.

If a print job stops responding, Mp10 Web refuses further printing from that computer until the helper is restarted, which is better than queueing work that is never going to come out. Somebody with administrator access can do that from **Admin → Print helper** in the menu, without leaving the browser; that page also lists which printers the helper can actually see, which is what solves most problems.

The full account — what each message means, how the helper is installed and configured, and how to work through a fault — is in the Mp10 Web — Printing guide. It is not repeated here, so that there is only ever one version of it to keep up to date.

1.18 Admin → Signature helper

The companion to **Admin → Print helper**, for the program that drives the signature pad and the scanner. Both live under the **Admin** heading in the menu, which starts collapsed.

It answers the two questions the greyed **Scan** and **Capture on pad** buttons raise, without anyone leaving the browser:

- **Is the helper running at this desk?** The top of the page shows its state, the build it is running, whether it is a service, which account it runs as, when it started, and its port.
- **Will it accept requests from this site?** *Allowed origins* lists the addresses it has been told to trust. If the address you use is not in that list, every request is refused — and a refused request looks exactly like a helper that is not running. This is the first thing to check.
- **What can this desk scan?** The **Scanners** card lists each scanner the helper can see, with its resolutions, colour modes, and whether it has a document feeder or can scan both sides. **Re-detect** looks again — use it after plugging a scanner in, rather than restarting anything.

If the Scanners card is empty, the helper genuinely cannot see a scanner; that is a driver or cabling question for IT, not something this screen can fix.

Setting the helper up is an IT task, covered in the **Mp10 — Signature Helper** guide.

1.19 Who made a change

Every change made through Mp10 Web is recorded under the web user's own operator code, the same way desktop changes are recorded under a desktop operator code. This means a colleague using the desktop application can see that a change came from you, from the web app, just like they would for any other change.